

PATH Questions

Methods

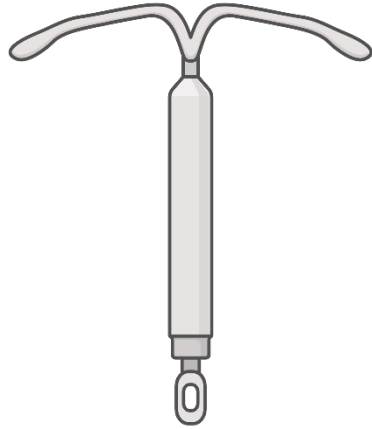
Contraindications

Emergency Use

Cases

Contraception Counseling

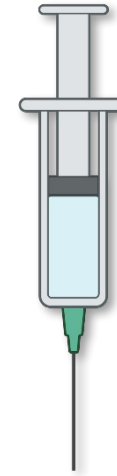
IUD



Implant



Depo-Provera



Patch



Pill



Ring



Condom



Contraception App



Bonus Resources

PATH Questions

Methods

Contraindications

Emergency Use

Cases

Bonus Resources

What phrasing do you use to discuss contraception options with your patients?

Conversation

opener

Pregnancy
Attitudes?

“Do you think you might like
to have (more) children at
some point?”

Timing?

“When do you think
that might be?”

How important
is
prevention?

“How important is it to you to
prevent pregnancy (until
then)?”

Method Selection

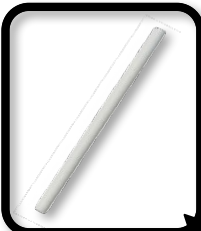
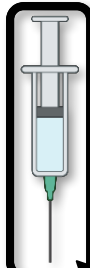
“What’s the most important
thing you’d like your birth
control to do for you?”

“What have you tried
in the past?”

“What worked well and
what did not?”

PATH Questions
Methods
OCP selection
Contraindications
Emergency Use
Cases
Bonus Resources

Methods of Contraception

						
	IUD		Implant	Depo-Provera	Patch	Pill
	Copper	LNG				
Hormone-Free						
Efficacy How many people will get pregnant? (per 100 users/y)	 <1	 <1				
Low Maintenance						
	Q3-10 years	Q3 years	Q3 month	Q week	Q day	
Impact on Bleeding	 Regular menses				 Can have regular menses	 Can have regular menses

PATH Questions
Methods
OCP selection
Contraindications
Emergency Use
Cases
Bonus Resources

Choosing an OCP



Estrogen

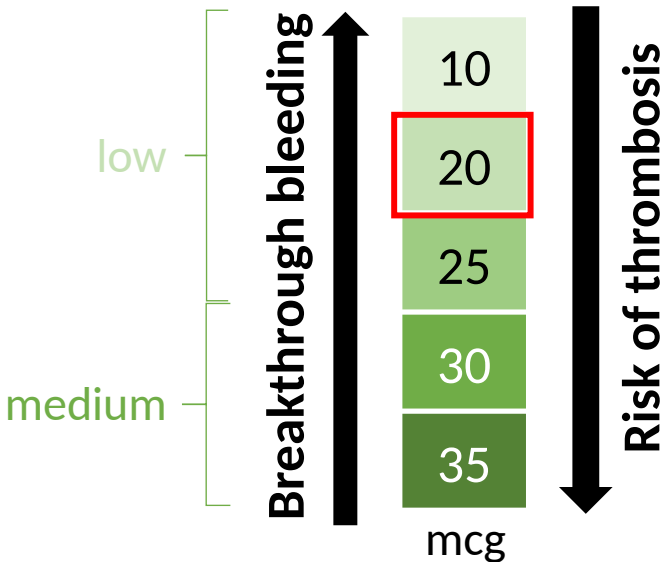
Suppresses ovulation
Stabilizes the endometrium

Progestin

Thickens cervical mucus to inhibit sperm migration
Endometrium less suitable for implantation

Mechanism

Ethinyl estradiol
(usually)



Generation

1	Norethindrone Ethinodiol	Risk of thrombosis when used with estrogens
2	Levonorgestrel Norgestrel	
3	Desogestrel Norgestimate	
4	Drospirenone	

Anti-androgenic

Ethinyl Combination OCP dosing progestin

PATH Questions

Methods

Contraindications

Emergency Use

Cases

Bonus Resources

Which methods are contraindicated with the following conditions?

Use the Contraception App: Medical Eligibility tab

**Smoking
Age >35**



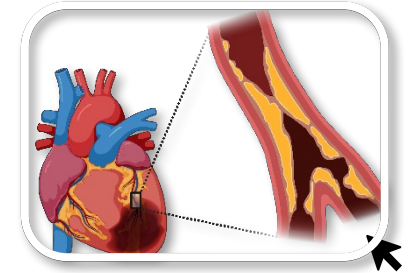
Avoid Combined
Hormone Contraception
(Pill, patch or ring)

Migraines with Aura



Avoid Combined
Hormone Contraception
(Pill, patch or ring)

**Personal history
of VTE/ CVD/ CVA**



Avoid Combined
Hormone Contraception
(Pill, patch or ring)

History of Breast Cancer



Avoid ANY hormone-based therapies

**Cervical or uterine cancer or
anatomic abnormalities or active PID**



Avoid IUDs

PATH Questions

Methods

Contraindications

Emergency Use

Cases

Emergency Use Contraceptives

Levonorgestrel
(Plan B)

Ulipristal
(Ella)

Copper
IUD

Combo
OCPs

	Levonorgestrel (Plan B)	Ulipristal (Ella)	Copper IUD	Combo OCPs
S afety	✓	Pump and discard if breastfeeding	Risk of perforation, infection, and ectopic	✓
T olerability	Headache, nausea, next menses irregular	Headache, nausea, next menses irregular	Initial spotting, then heavier menses	↑ nausea/vomiting, headache, next menses irregular
E fficacy	52-100% if <72h. Less effective if 72-120h or BMI >25	62-85% if <120h. Less effective when on progestin methods	99.9% effective if <5 days	50% if <120h
P rice	\$\$ \$35-50 each use	\$\$ \$50-60 each use	\$\$\$\$ \$500-1,000	\$ 4-5 pills of generic OCP pack
S implicity	PO within 120h of intercourse	PO within 120h of intercourse	Intrauterine insertion q10 years	PO q12h x2 doses <120h of intercourse

Contraception App



Adapted from Contraception app
developed by Dr. Joshua Steinberg

Bonus Resources

PATH Questions

Methods

Contraindications

Emergency Use

Cases

1-2

3-4

Bonus Resources

Case 1

Ella is a 23 year-old F with history of acne who comes to clinic to establish care. She is interested in starting a combined oral contraceptive pill (COCP) to prevent pregnancy.

Vitals and physical exam are normal. BMI is 35.

What category risk are COCPs for this patient?

Category 2 due to BMI > 30 (advantages generally outweigh theoretical or proven risks)

1	Use without restriction
2	Advantages > Risks
3	Usually not recommended
4	DO NOT USE

How will you counsel her on benefits/risks?

Assess contraindications to estrogen (tobacco use, migraine with aura, history of VTE). Counsel on possible **reduced efficacy due to BMI** and don't protect against STIs.

What dose of estrogen /generation(s) of progestin would be most suitable?

Low to medium dose estrogen. For her acne, it may be beneficial to use 3rd or 4th G progestins.

How can she skip her period if she plans to travel?

Skip placebo pills and continue the next pack.

Case 2

Miranda is a 35 year-old F with well-controlled hypertension, bipolar depression on lamotrigine, and menorrhagia who is interested in switching her COCP to another contraception method for pregnancy prevention.

What category risk is associated with lamotrigine and combined hormonal methods?

Category 3 – not recommended unless no other options. Estrogen ↑ lamotrigine clearance.

1	Use without restriction
2	Advantages > Risks
3	Usually not recommended
4	DO NOT USE

What options would be discreet?

IUD, Depo-Provera, or Nexplanon (implant)

How can you counsel her on choosing between levonorgestrel IUDs?

Duration (3, 5, 6, or 7 years) and **period** preferences (interest in having lighter vs no period)

How can you counsel her on benefits, risks, and side effects?

Benefits: efficacy, lighter/no period
Risks: procedural complications, STIs
Side effects: irregular bleeding 3-6 mos

PATH Questions

Methods

Contraindications

Emergency Use

Cases

1-2

3-4

Bonus Resources

Case 3

David is a 21 year-old transgender man with obesity and type I diabetes who presents for “the shot”. He is sexually active with one male partner.

His last HbA1C was 9% and he was found to have microalbuminuria and mild retinopathy during a prior visit.

What categories of risk are associated with his condition?

He has DM with complications. Combined hormonal methods and Depo-Provera are category 3. All others (except for copper IUD) are category 2.

What methods of contraception may worsen gender dysphoria in a female-to-male transgender patient?

Avoid methods that would worsen menstrual bleeding (e.g., copper IUD). Consider methods that eradicate or lighten menstrual bleeding (e.g. LNG IUD).

In addition to helping him select an alternative method, what additional counseling will you provide?

Counsel on condom use for STI prevention.
Use the PATH questions to assess future pregnancy goals.

Case 4

Kylee is a 35 year-old woman who presents for contraceptive counseling. What options for combined hormonal contraceptives are recommended if she had the following comorbidities?

Compensated cirrhosis from hepatitis B

All methods would be low risk

Decompensated cirrhosis from alcohol

COCs would be contraindicated. All other methods except copper IUD are category 3.

Smoking

For combined hormonal methods:

- < 35 yo – category 2
- > 35 yo, smoking <15 cigarettes/day – category 3
- > 35 yo, smoking > 15 cigarettes/day – category 4.

All other methods are safe.

...You prescribe the patch. She breaks her leg in a skiing accident and requires ORIF.

How will you manage her contraception after surgery?

Consider switching to a progestin only method. Combined hormonal methods are category 4 after major surgery with anticipated immobilization.

PATH Questions

Methods

Contraindications

Emergency Use

CASES

Bonus Resources

Resources for Clinicians and Patients

- [bedsider.org](https://www.bedsider.org): A free website developed by the National Campaign to Prevent Teen and Unplanned Pregnancy, a private nonprofit group
- [The Family Planning National Training Center](https://www.familyplanning.org): The website for federally funded contraceptive resources developed with the support of the Office of Population Affairs
- [Center for Young Women's Health](https://www.childrenshospital.org/centers/young-womens-health): A free website run by Boston Children's Hospital that addresses reproductive health needs of teens and young adults
- [Beyond the Pill](https://www.beyondthepill.org): A free website run by the University of California San Francisco
- [ReproductiveAccess.org](https://www.reproductiveaccess.org)
- [SexualityandU.ca](https://www.sexualityandU.ca): An educational site run by the Society of Obstetricians and Gynaecologists of Canada that includes descriptions of various methods and a tool to help with selection of birth control
- [Planned Parenthood](https://www.plannedparenthood.org): A nonprofit organization dedicated to reproductive health with resources for patients and clinicians
- [ACOG Contraceptive FAQs](https://www.acog.org/clinical/clinical-guidance/faq): American College of Obstetricians and Gynecologists addresses frequently asked questions (FAQs) about contraception
- [ACOG LARC Program](https://www.acog.org/clinical/clinical-guidance/larc-program): American College of Obstetricians and Gynecologists Long-Acting Reversible Contraception Program
- [United States Medical Eligibility Criteria for Contraceptive Use](https://www.medicineskills.com/contraception)
- [United States Selected Practice Recommendations for Contraceptive Use](https://www.medicineskills.com/contraception)
- [World Health Organization Medical Eligibility Criteria for Contraceptive Use](https://www.medicineskills.com/contraception)

2024 Practice Update — CDC US MEC & US SPR (added in optimization)

CDC released updated US MEC 2024 and US SPR 2024 (MMWR, Aug 2024) — this deck's framework still holds; key additions below.

- US MEC 2024: new recommendations for chronic kidney disease; revised guidance across DVT/PE ± anticoagulation, thrombophilia, SLE, cirrhosis/liver tumor, sickle cell, solid-organ transplant, obesity, surgery, and ARV drug interactions; new formulations added (COCs, patch, ring, POPs, LNG-IUDs, vaginal pH modulator).
- NEW transgender guidance (directly relevant to Case 4/David): testosterone does NOT reliably prevent pregnancy — up to ~36% of users ovulate. Offer contraceptive counseling to testosterone users with a uterus who do not desire pregnancy.
- US SPR 2024: lidocaine (paracervical block or topical) recommended to reduce pain at IUD placement; new support for self-administration of subcutaneous DMPA (DMPA-SC 104 mg).
- OTC access: Opill (norgestrel 0.075 mg, progestin-only) — first FDA-approved nonprescription daily oral contraceptive (FDA Jul 2023; on shelves 2024, ~\$20/mo). No estrogen; same POP cautions (e.g., current breast cancer).
- Newer non-hormonal method: vaginal pH modulator gel (Phexxi; lactic/citric acid + potassium bitartrate; FDA 2020) — typical-use efficacy ~86% (per-cycle), used immediately before intercourse.

Sources: CDC US MEC 2024 (MMWR RR-73/4); CDC US SPR 2024 (MMWR RR-73/3); FDA Opill approval 2023; Phexxi label/AMPOWER (Obstet Gynecol 2020). Educational use — clinician-reviewed.

Take-Home Points

- **Lead with the patient: use open-ended PATH questions to gauge pregnancy attitudes/timing; avoid efficacy-first or assumption-laden counseling.**
- Match the method to what matters to the patient — hormone-free vs hormonal, efficacy, maintenance burden, and effect on bleeding all drive the choice.
- Take a focused history for estrogen contraindications: smoking at age >35, migraine with aura, VTE/CVD/CVA, breast cancer. When unsure, use the US MEC categories (1–4) in the Contraception app.
- A reasonable COC default is ethinyl estradiol ~20 mcg + a progestin; reserve 3rd/4th-gen progestins for anti-androgenic benefit given higher VTE risk. Have 2–3 go-to pills.
- Emergency contraception: copper IUD is most effective; ulipristal (Ella) and levonorgestrel (Plan B) balance efficacy and tolerability; combined OCPs (Yuzpe) are the widely-available backup.
- Know the 2024 updates: testosterone is NOT contraception; lidocaine for IUD-placement pain; self-administered DMPA-SC; and OTC Opill expand access.

Attribution & Optimization Notes

- **Original: “Contraception” — TeachIM.org, published April 2022.**

- Authors: deQuillfeldt, Saxon, Guinn, Ramaswami, Haynes, Cleeves, Soiseth, Fainstad, Do; Section Eds. Brett, McClintock; Exec. Ed. Zhang.
- Source URL: https://teachim.org/teaching_material/contraception/

- **License: CC BY-NC 4.0 — attribution required, non-commercial use only. Optimized June 2026.**

Corrections made to this deck (surgical, animations preserved):

- Slide 7 MEC legend: “Usual not recommended” → “Usually not recommended” (×2).
- Slide 8 Case 3: “In additional to” → “In addition to”.

Added: 2024 CDC US MEC/US SPR practice-update slide; take-home slide. No clinical thresholds changed from memory — all updates source-cited.

Companion files (copied into lesson folder): Facilitator Guide, Learner Handout, Contraception Cases (learner + facilitator/answer key), PATH Questions handout.